

Women's Health

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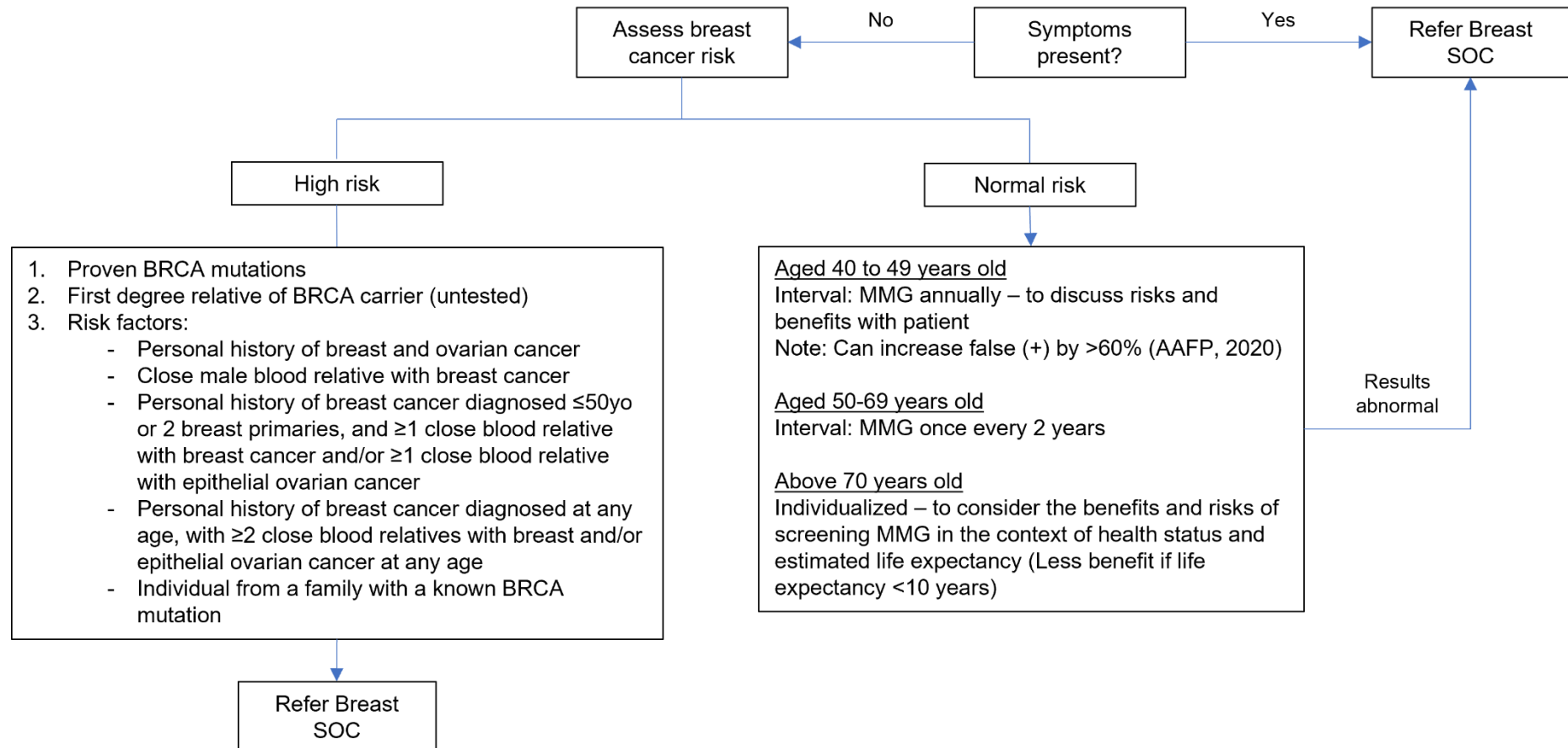
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PREVENTIVE HEALTH

Breast Cancer Screening



For high risk patients, screening starts at 25 years old for proven carriers without family history, or 5-10 years earlier than the youngest family member with breast cancer.

All women ≥ 30 years old are recommended for monthly Breast Self-Examination (BSE) to improve women's awareness of their own breast changes and breast cancer.

**Mammogram is not done in NUP Polyclinics for women with breast implants – please refer to the assessment centres for screening under Breast Screen Singapore.*

Criteria for Direct Access / Fast Track Referral to Breast Clinic

No.	Presentation	Remarks
1	Suspicious mammogram or ultrasound finding of BIRADS 4/5 categories	Abnormal reports from screening mammogram or ultrasound
2	Suspicious symptoms and signs	• Hard, fixed and irregular lumps in women 40 years and above (<i>at any age for NUH</i>) • Bloody nipple discharge
3	Patients newly diagnosed with breast cancer	Diagnosis made by private GP or specialist
4	Clinical diagnosis of breast abscess	To use clinical judgment in determining if patient requires ED referral instead of urgent referral to breast clinic, e.g. fever, large abscess.
5	Patients recalled by Breast Screen Singapore (BSS) but would like to be managed out of BSS	Patients screened under BSS are strongly encouraged to obtain their appointments under BSS

Notes

1. The hospitals which accept these referral criteria for direct access / fast track appointments are NTFGH, NUH, SGH, NCCS, KKWCH, CGH and TTSH.
2. To facilitate care integration and enhance care transition, please consider referring within NUHS institutions, i.e. NTFGH, AH and NUH
3. Additionally, NUH accepts direct access referrals for patients presenting with hard, fixed, and irregular breast lumps regardless of age.
4. Patients presenting with breast conditions outside of these criteria may be referred routinely as a non-urgent referral.
5. Patients who present with breast conditions that do not fulfil these criteria but are deemed clinically to need to be seen earlier, may be referred to the breast clinic as an early appointment.

Cervical Screening Singapore (CSS)

What Screening Tests

- Age 25 - 29 years: Cervical cytology
- Age 30 - 69 years: HPV DNA test

Who to Screen

- All women who have ever had sex or are sexually active are advised to have their first Pap smear by age 25

When to Screen

- Cervical cytology: once every 3 years
- HPV DNA test: once every 5 years

When to Stop Screening

- A woman can be discharged from screening at 69 years of age if she has:
 - 3 consecutive negative cervical cytology tests OR
 - 2 consecutive negative HPV tests in the last 10 years, with the most recent test occurring within last 5 years OR
 - 2 consecutive negative Co-tests in the last 10 years, with the most recent test occurring within the last 5 years
- For women who had history of CIN2, CIN3 or AIS, routine screening should continue for at least 20 years, even if it extends beyond 69 years of age

Special Populations

- Immunocompromised women (HIV positive, undergone solid organ transplant, or clinical conditions requiring 2 or more immunosuppressive agents):
 - Annual cervical cytology for women between 25 to 29 years old
 - 3 yearly HPV primary screening for women who are 30 years old and above:
 - Those who are tested with any high risk HPV strains should be sent for colposcopy instead of doing a cytology triage
 - Lifetime screening
- Women who have undergone hysterectomy
 - Subtotal hysterectomy: Routine screening as per guidelines
 - Benign condition of uterus and not cervical cancer precursors/cancer: Stop screening
 - Unknown histology: Do 1 baseline vault smear, stop if negative
 - Past history of invasive gynaecological cancer, or previously treated for vaginal intra-epithelial neoplasia: Should be followed up with gynaecologist
 - Past history of CIN:
 - If excision margin was involved or not adequately assessed histologically: Follow up should be at the discretion of the gynaecologist. In general, vault smear at least yearly.
 - CIN 1/2/3 completely excised during hysterectomy: Vault smear for five years yearly, then 2-yearly subsequently.

** Patients who require a vault smear will not be eligible to participate in the CSS program.*

Human Papillomavirus Vaccination

Who to Vaccinate

Females aged 9 - 26 years old*

Bear in mind: It is most effective if given before first-ever sexual exposure; however, if female is already sexually active, they may still benefit from the vaccine as they may not have been exposed to the HPV subtypes that are in the vaccinations detailed below.

Types of HPV Vaccines Available in Singapore

Brand Name	Cervarix [^]	Gardasil	Gardasil 9
Offers protection against	HPV 16, 18 (high risk strains)	HPV 6, 11 (low risk strains) 16, 18 (high risk strains)	6, 11 (low risk strains) 16, 18, 31, 33, 45, 52, 58 (high risk strains)
Approved indications	Prevention of cervical cancer and premalignant cervical lesions caused by HPV 16 and 18.	Prevention of cervical, vulvar, vaginal and anal cancer caused by HPV types 16 and 18. Prevention of premalignant cervical, vulvar, vaginal and anal lesions caused by HPV types 6, 11, 16 and 18. Prevention of genital warts caused by HPV types 6 and 11.	Prevention of cervical, vulvar, vaginal and anal cancer caused by HPV types 16, 18, 31, 33, 45, 52 and 58. Prevention of dysplastic, premalignant cervical, vulvar, vaginal and anal lesions caused by HPV types 6, 11, 16, 18, 31, 33, 45, 52 and 58. Prevention of genital warts caused by HPV types 6 and 11
Approved age for use	Females 9 to 25 years old, or as advised by doctor.	Females aged 9 to 26 years old, or as advised by doctor.	Females aged 9-26 years old, or as advised by doctor.
Vaccination schedule	1) Females 9 - 14 years old: 2 doses, 5 - 13 months apart. 2) Females aged 15 - 25 years old: 3 doses: 2 nd dose at 1 - 2.5 months after first dose; 3 rd dose at 5 - 12 months after the first dose.	1) Females 9 - 14 years old: 2 doses, 6 - 12 months apart. 2) Females aged 15 - 25 years old: 3 doses at 0, 2 and 6 months.	1) Females 9 - 14 years old: 2 doses, 6 - 12 months apart. 2) Females aged 15 - 25 years old: 3 doses at 0, 2 and 6 months.

Table from <https://www.healthhub.sg/a-z/diseases-and-conditions/701/faqs-on-hpv-and-hpv-immunisation>

[^] Take note that only Cervarix is currently available at NUP.

* The American College of Obstetricians and Gynecologists (ACOG) also recommends that for some women aged 25-45 years who are previously unvaccinated, physicians may use shared clinical decision making regarding the HPV vaccination, considering the patient's risk for acquisition of a new HPV infection and whether the HPV vaccine may provide benefit.

Risks of vaccination

- Pain, swelling, itching and redness at the site of the injection
- Fever
- Serious adverse events are rare.¹

Special Precautions

Not recommended during pregnancy; however routine pregnancy testing is not recommended before vaccination.¹

Can and should be given to breastfeeding women aged 26 years and younger who had not previously been vaccinated.¹

Immunocompromised states (e.g. HIV infection) are not a contraindication to the HPV vaccination.¹

Not to be given to those with yeast allergy (Gardasil / Gardasil 9)

Not recommended that an individual who had previously received the quadrivalent HPV vaccine (Gardasil) be revaccinated with the 9-valent HPV vaccine (Gardasil 9).¹

FAMILY PLANNING

Contraception Reference Chart

Condition ¹		COC	DMPA	Cu-IUD
Age	Menarche to 49 years	√	√	√
	Below 18 years of age	√	*	*
Nulliparous		√	√	√
Breastfeeding ²	Less than 6 weeks post-partum	X	√	*
	More than 6 weeks post-partum	√	√	*
Smoking	Age < 35 years	√	√	√
	Age ≥ 35 years	X	√	√
Obesity	BMI ≥ 30 - 34 kg/m ²	√	√	√
	BMI ≥ 35kg/m ²	X	√	√
Hypertension	Systolic BP ≥140 or Diastolic BP ≥ 90	X	√	√
	Vascular disease	X	*	√
Ischemic heart disease or stroke		X	X	√
Diabetes without complications		√	√	√
Diabetes with retino / nephron / neuropathy		*	√	√
Known hyperlipidaemia		√	√	√
Severe headaches / migraine without aura		X	√	√
Migraine with aura		X	√	√
History of deep vein thrombosis or pulmonary embolism		X	*	√
Varicose veins		√	√	√
Thyroid disease		√	√	√
Thalassemia		√	√	*
Hepatitis	Active	X	√	√
	Carrier	√	√	√
Liver disease	Cirrhosis (Mild)	√	√	√
	Cirrhosis (Severe)	X	*	√
	Tumours	X	X	√
Depressive disorder		√	√	√
Breast disease	Undiagnosed mass	X	*	√
	Family history of cancer	√	√	√
	Current cancer	X	X	√
Uterine fibroids without cavity distortion		√	√	√
Endometriosis ³		√	√	√
Cervical ectropion		√	√	√
Cancer	Cervical	√	√	X
	Endometrial	√	√	X
	Ovarian	√	√	√
Vaginal bleeding	Irregular without heavy bleeding	√	*	√
	Heavy or prolonged (regular / irregular)	√	*	*
	Unexplained bleeding	X	X	X
STIs / PID		√	√	X

COC - Combined Oral Contraceptive

DMPA - Depot Medroxyprogesterone Acetate (Progesterone-only)

Cu-IUD - Copper Intrauterine Device

Legend:

√ There are no restrictions for use

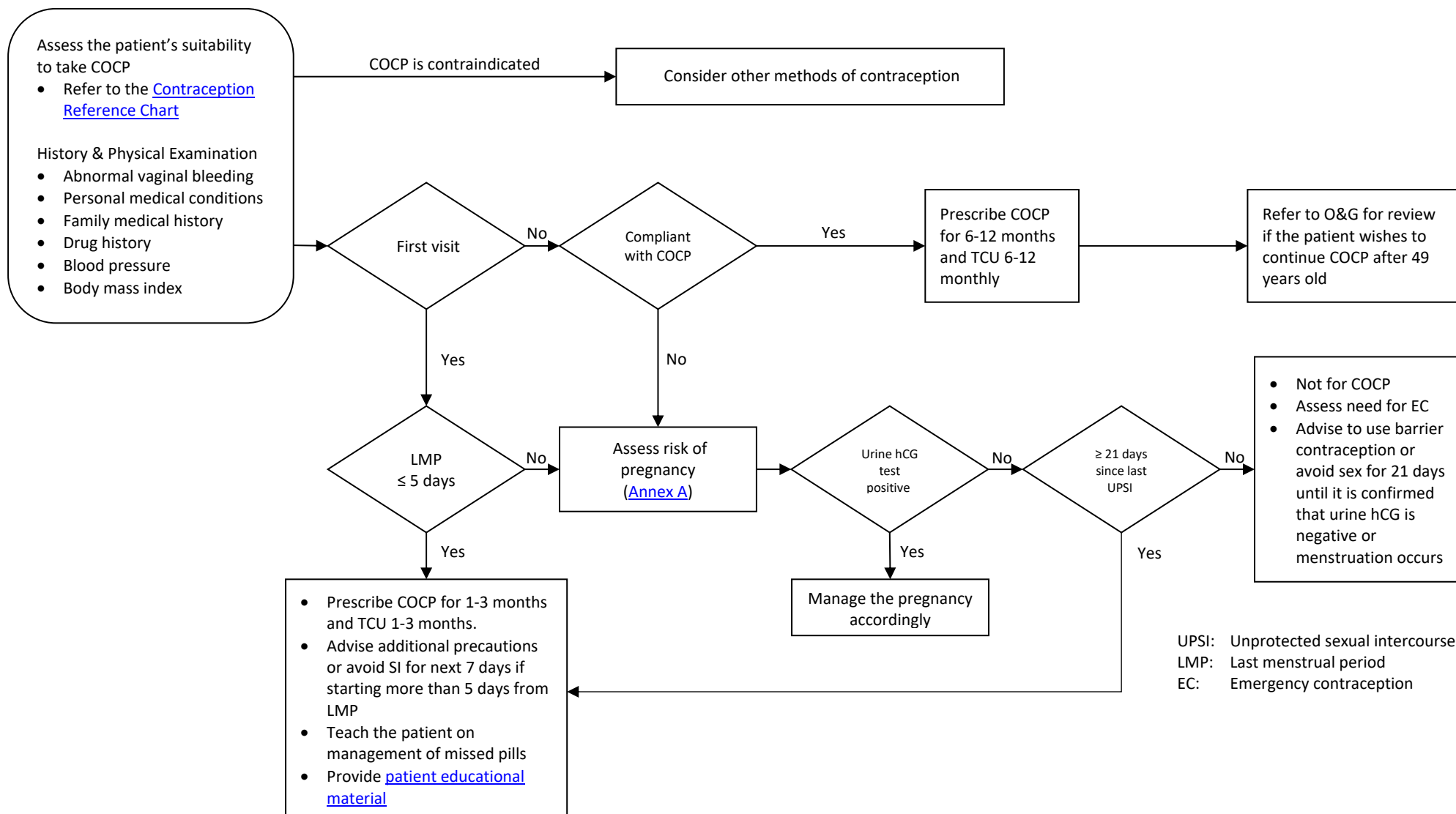
* Clinical judgment is required for use, some follow-up may be needed

x The method should not be used

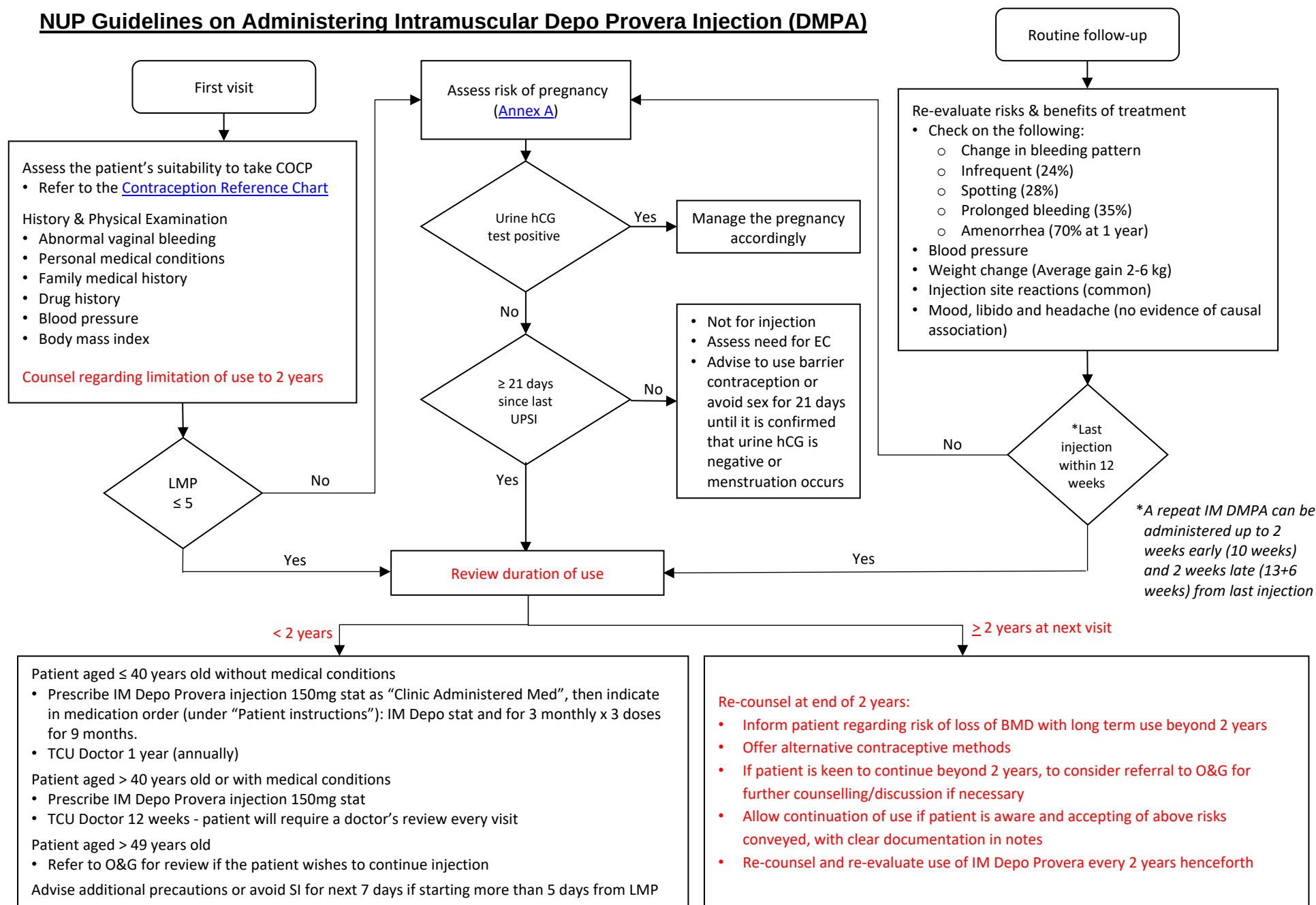
Footnotes / Special Considerations:

1. For Cu-IUD, please assess clinically for possible anaemia (e.g. history of menorrhagia, documented low Hb previously) KIV check FBC before Cu-IUD insertion and advise accordingly.
2. In patients who are postpartum and breastfeeding, there is increased risk of perforation up to 35 weeks postnatal – IUD insertion should be undertaken by experienced fitter
3. IUS preferred for endometriosis. Adenomyosis increases risk of expulsion.
4. IM Depo Provera should not be used beyond 2 years without proper counselling on risks and benefits

NUP Guidelines on Prescribing Combined Oral Contraceptive Pills (COCP)



NUP Guidelines on Administering Intramuscular Depo Provera Injection (DMPA)



NUP Guidelines on Administering Intramuscular Depo Provera Injection (DMPA)

For appropriate counselling at initiation and every 2 years use of IM Depo Provera

FDA black box warning (2004)

- Women who use Depo Provera may lose significant bone mineral density. Bone loss is greater with increasing duration of use and may not be completely reversible.
- It is unknown if use of Depo Provera during adolescence or early adulthood, a critical period of bone accretion, will reduce peak bone mass and increase the risk of osteoporotic fracture in later life
- Depo Provera is not recommended as a long term (i.e. longer than 2 years) birth control method unless other options are considered inadequate

However, later evidence has shown:

- Use of IM Depo Provera is associated with a small loss of BMD, which is usually recovered after discontinuation
- There is insufficient data to determine if BMD changes lead to increased fracture risk with long term use

International guidelines since the black box warning was issued:

WHO (2005)

- There should be no restriction on use of DMPA in women 18-45yo, including no restriction on duration of use
- For women <18yo and >45yo, advantages of using DMPA outweigh theoretical safety concerns regarding fracture risk

UK FSRH (2014)

- In women <18yo, DMPA can be used after consideration of alternative methods of contraception
- Women using DMPA who wish to continue use should be reviewed every 2 years for discussion of benefits and potential risks

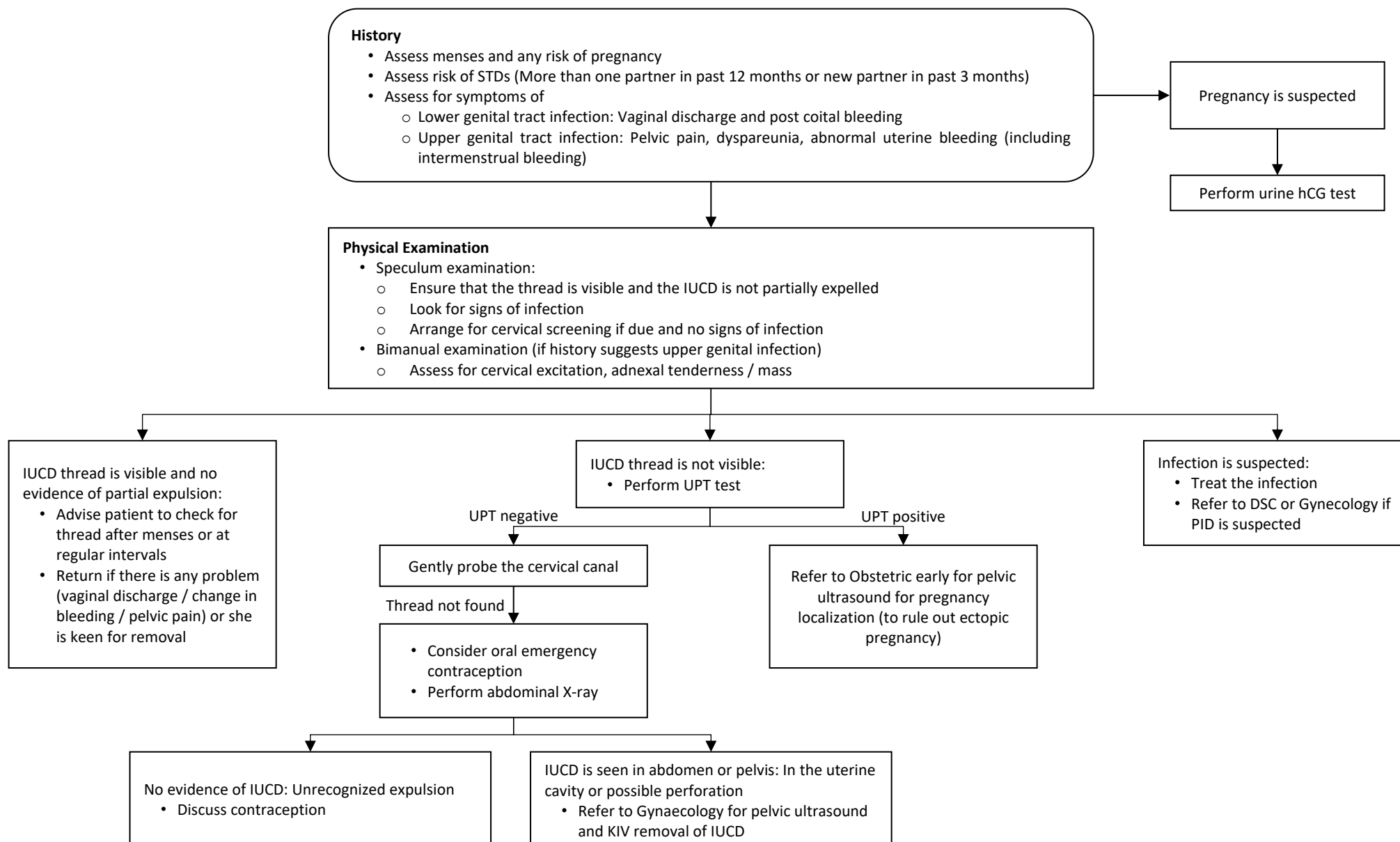
ACOG

- Healthcare providers should inform women and adolescents considering initiating DMPA or continuing the method about the benefits and risks and should discuss the FDA black box warning
- Concerns regarding the effect on DMPA on BMD and potential fracture risk should not prevent practitioners from prescribing DMPA or continuing use beyond 2 years
- Potential health risks associated with the bone effects of DMPA must be balanced against the public health concerns of unwanted pregnancies, especially among adolescents

Hence, the use of IM Depo Provera in NUP is recommended to be up to 2 years. For patients who are keen to continue beyond 2 years after appropriate counselling of risks and benefits, they should be re-evaluated and counselled every 2 years hence forth.

The date of initiation and date of counselling (at 2 year mark, and after) are to be clearly documented in Care Coordination notes, in addition to the documentation of counselling of risks in patient's notes.

NUP Guidelines on Post Intrauterine Contraceptive Device Insertion - Follow-up Visit at 6 Weeks and 6 Months ***(5-year Copper IUCD is available at NUP)***



Contraceptive Methods Available in NUP – Comparison Chart

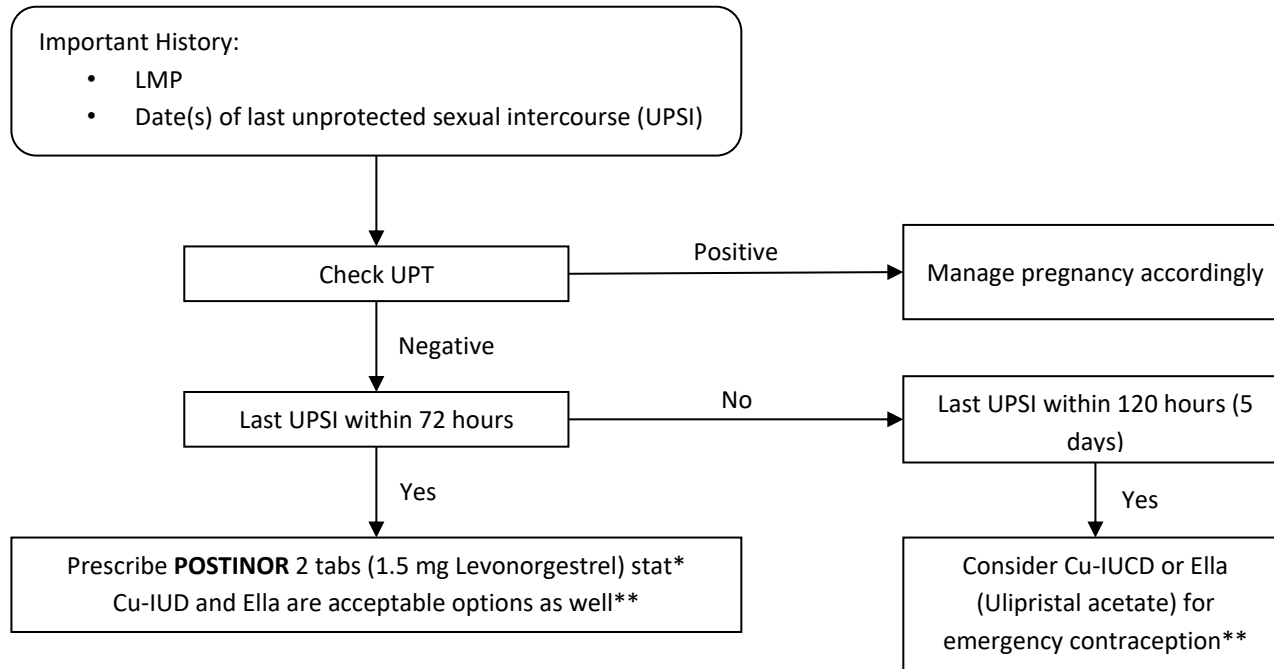
	COCP	DMPA	Cu-IUCD
Pros	- Makes menses more regular, with lighter flow - Reduces menstrual cramps - Does not interrupt sexual activity - May reduce symptoms of PMS - Protects against ovarian, uterine and colorectal cancer	- No or scanty menses while on regular injections - Protects against uterine cancer - Does not interrupt sexual activity	- Provides protection once inserted, does not interrupt sexual activity - Does not require daily attention
Cons	- Does not protect against STIs - Needs to be taken at same time every day - Certain medical conditions are contraindications to COCP use - Slightly increases risk of cervical and breast cancer, though risk reduces over time upon cessation of use	- Does not protect against STIs - Needs to see a healthcare provider every 3 months for the injection - Some women get irregular menses or spotting for the first 3-6 months or longer - Side effects: weight gain, mood changes, headaches, sore breasts - Black box warning on effect on BMD leading to limitation of use to 2 years	- Does not protect against STIs - Needs to be inserted by a healthcare provider in an outpatient setting - Slightly higher risk of infection in the initial period after insertion - Risk of IUCD falling out - Risk of procedure: perforation - Side effects: increased menstrual cramps, heavier and longer menses
Failure Rate with Typical Use (% of women with an unintended pregnancy in the first year of use)	9%	6%	0.8%

Alternative Contraceptive Methods Not Available in NUP

Good to know to advise patients on all options – to refer to SOC if patients are keen

Hormonal IUD (Mirena)	Progestrone-only (contains Levonorgestrel) Useful for women with menorrhagia and dysmenorrhea - helps alleviate such symptoms due to progesterone effects Treats endometriosis / adenomyosis / endometrial hyperplasia Effective for 5 years
Implanon	Progestrone-only (contains Etonogestrel) Subdermal, non-biodegradable single sterile rod implant - does not contain latex, radio-opaque (4 cm in length, 2 mm in diameter) Effective for 3 years Contraceptive effectiveness of the ENG-IMP is not affected by body weight or BMI Complications of insertion: • Inserted too deeply (intramuscular or in fascia), neural or vascular injury may occur • Migration of implant • Non palpable due to encasement in fibrous tissue or weight gain Refer to Gynaecology for assessment if not palpable
Evra Patch	Combined oestrogen and progesterone (contains Ethinylestradiol and Norelgesetromin) Applied once a week for 3 weeks, then stop for 1 week Recommended for application on clean, dry and hairless skin (e.g. buttock, abdomen, upper outer arm or upper back) Avoid open wounds, cuts or irritated skin To change site of application for each new patch
Tubal Ligation	Permanent, irreversible method Done under general anaesthesia

Emergency Contraception



* Divided dosing of Postinor increases failure rate as patients forget to take the second dose. Hence single stat dosing preferred.

** Both Cu-IUD as emergency contraception and Ella are not available in NUP - please refer as necessary.

Common side effects of Postinor:

- Nausea, vomiting
- Headache
- Abdominal cramps

Postinor has 85% effectiveness in preventing expected pregnancy.

Do remember to also counsel about safe sex practices and discuss regarding regular contraception.

ANTENATAL CARE

Objectives of Antenatal Visits:

1. Assess maternal and foetal wellbeing.
2. Identify risk factors and consider appropriate interventions.
3. Provide advice, assurance, education and support for the patient and her family.
4. Offer Down Syndrome screening for all women at any maternal age.
5. Review all investigation results promptly to ensure normality or act appropriately if the results are abnormal.

Gestation	History	Examination and Investigations	Plan
< 12 weeks	<u>History taking</u> • LMP • Current gestation by LMP • Maternal age • Obstetric and gynaecological history • Past medical history • Family history • Lifestyle: Smoking, alcohol • Symptoms in current pregnancy High risk pregnancy - Refer to Obstetric	<u>Physical examination</u> • Parameters (height, weight, BP) • Auscultate heart and lungs • Breast exam • Abdominal exam <u>Routine investigation at each visit</u> • Urine dipstick test (glucose, protein)	Explain and offer: <u>To be done at NUH</u> • Dating scan • First Trimester Screening (11-13+6 weeks) or non-invasive prenatal testing (NIPT) <u>To be done at NUP</u> • Antenatal blood tests (FBC, HBsAg, anti-HBs Ab, Syphilis, HIV, ABO and Rhesus status) - all included in antenatal panel* • Thalassemia screen for all pregnant women with unknown Thalassemia status • 2-point OGTT for patients at high risk for GDM as early as possible in pregnancy to screen for pre-existing DM and to repeat a 3 point OGTT at 24-28 weeks if the test is normal <u>Medication</u> • Folic acid 5 mg OM (Ideally to be started at least 3 months prior to conception) • If on levothyroxine for hypothyroidism, empirically increase dose by 30% (i.e. Double up the dose on 2 days each week) as soon as possible in pregnancy without awaiting TFT results and prior to referral to NUH clinic. • Review potentially teratogenic drugs. Stop/replace or seek advice, as appropriate. <u>Advice to patient</u> • Advise to go ED if she has abdominal pain or vaginal bleeding

* "Antenatal panel (without thalassaemia screen)": Ordered for pregnant women with a known thalassaemia status

"Antenatal panel (with thalassaemia screen)": Ordered for pregnant women who do not know their thalassaemia status

Gestation	History	Examination and Investigations	Plan
≥ 12 weeks * 1st visit	<u>History taking</u> • LMP / EDD • Current gestation • Maternal age* • Obstetric and gynaecological history* • Past medical history* • Family history* • Lifestyle: Smoking • Symptoms in current pregnancy • Review results of previous investigations and scans at NUP and SOC Assess risk factors for pre-eclampsia (Please refer to #5 in <u>Special Problems in Pregnancy</u>) and offer low dose aspirin 100mg every NIGHT to commence between 12-16 weeks' gestation and continued up to 36 weeks. High risk pregnancy - Refer to Obstetric	<u>Physical examination</u> • Parameters (height, weight, BP) • Auscultate heart and lungs • Abdominal exam • Symphyseal fundal height • Doptone (foetal heart beat) <u>Routine investigation at each visit</u> • Urine dipstick test (glucose, protein)	Explain and offer: <u>To be done at NUH</u> • Dating scan* • Non-invasive prenatal testing (NIPT)* • Screening scan for foetal abnormalities at 18-22 weeks <u>To be done at NUP *</u> • Antenatal blood tests (FBC, HBsAg, anti-HBs Ab, Syphilis, HIV, ABO and Rhesus status) • Thalassaemia screen for all pregnant women with unknown Thalassaemia status • 3-point OGTT at 24-28 weeks for all pregnant women <u>Medication</u> • Obimin 1 tab OM (or other prenatal multivitamin) <u>Advice to patient</u> • Advise to go ED if she has abdominal pain or vaginal bleeding
≥ 20 weeks * 1st visit (late booker)	<u>History taking</u> • LMP / EDD • Current gestation • Maternal age* • Obstetric and gynaecological history* • Past medical history* • Family history* • Lifestyle: Smoking • Symptoms in current pregnancy • Foetal movement • Review results of previous investigations and scans at NUP and SOC <i>*All pregnant women presenting at ≥ 20 weeks gestation at first visit: Refer to Obstetric early</i>	<u>Physical examination</u> • Parameters (height, weight, BP) • Auscultate heart and lungs • Abdominal exam • Symphyseal fundal height • Doptone (foetal heart beat) Refer to Obstetric if SFH is larger or smaller than dates by 2cm <u>Routine investigation at each visit</u> • Urine dipstick test (glucose, protein)	Explain and offer: <u>To be done at NUH</u> • Screening scan for foetal abnormalities at 18-22 weeks • Growth scan (28-32 weeks) <u>To be done at NUP</u> • 3-point OGTT at 24-28 weeks for all pregnant women • Arrange for vaccination: influenza and Tdap (ideally between 20-28 weeks) <u>Medication</u> • Obimin 1 tab OM (or other prenatal multivitamin) <u>Advice to patient</u> • Advise to go ED if she has abdominal pain, vaginal bleeding, leaking liquor or decreased foetal movements

Investigations at SOC

6-12 weeks: Dating scan
11-13+6 weeks: Nuchal translucency and first trimester screen, or NIPT
18-22 weeks: Fetal abnormality scan
28-32 weeks: Growth scan

Frequency of Antenatal Follow-up at NUP

Less than 28 weeks of gestation: 4 weekly
More than 28 weeks of gestation: 2 weekly
More than 32 weeks of gestation: Refer to Obstetric to continue follow-up

Women are at high risk for GDM if they have:

1. BMI 25 kg/m² or more
2. Age 40 years or older
3. First-degree relative with Diabetes
4. Personal history of previous GDM, pre diabetes or large babies > 4 kg
5. Personal history of polycystic ovarian syndrome
6. Previous poor obstetric outcomes usually associated with Diabetes
7. Glycosuria 2+ or more in current pregnancy

Perform 2 point OGTT for these patients in the first trimester and repeat a 3 point OGTT at 24-28 weeks if the initial test is normal.

Antenatal Investigation	Investigation Results
Full blood count	Note Hb and MCV Also consider iron studies if Hb < 11 g/dl, MCV < 80 fl
Thalassemia screening	Include Thalassemia screen for all pregnant women with unknown Thalassemia status Refer to Obstetric if newly diagnosed Thal KIV Thalassemia screen for spouse
HBsAg, anti-HBs Ab	If HBsAg negative and not immune, recommend Hep B vaccination postnatally If HBsAg positive, refer to Gastro for follow-up
Syphilis screening	If positive, refer to DSC and Obstetric
HIV screening	Refer to CDC if positive
ABO blood group and Rhesus status	Refer to Obstetric if Rh negative and partner is Rh positive Needs anti-D immunoglobulin prophylaxis
75 g OGTT	Refer to Obstetric if diagnosed with GDM or DM in pregnancy

Refer to ED stat if a pregnant woman presents with:

- Vaginal bleeding, abdominal pain of uterine origin, leaking liquor, and/or decreased fetal movement

High Risk Pregnancy – Screen and Refer at First Visit

Present Pregnancy

1. Age < 18 years or ≥ 35 years at time of delivery (EDD)
2. Uncertain dates
3. Late booking (presenting at ≥ 20 weeks gestation)
4. Primary or secondary subfertility
5. Substance abuse
6. Cervical incompetence
7. Any complicated pregnancy with a diagnosed condition by another healthcare professional

Past Obstetric History

1. Recurrent spontaneous abortion
2. Uterine rupture
3. Molar pregnancy
4. Grand multiparity (Parity ≥ 5)
5. Previous severe pregnancy induced hypertension
6. Low birth weight (< 2500 g)
7. Macrosomia (> 4000 g)
8. Previous infant with congenital or genetic disorders
9. Previous intrauterine death / stillbirth / early neonatal death
10. Previous Caesarean section

Medical or Surgical Conditions

1. Pre-existing hypertension
2. Pre-existing diabetes mellitus
3. Systemic diseases e.g. cardiac disease, thyroid disorders, autoimmune disorders, renal disease, liver disease
4. Epilepsy, previous cerebrovascular accident or central nervous system disorder
5. Past surgery e.g. myomectomy, pelvic and uncertain abdominal operations
6. Psychiatric disorders
7. Venous thromboembolism (e.g. Deep vein thrombosis, pulmonary embolism)
8. Fibroid (past or present)
9. Other chronic conditions that may potentially affect the mother and/or fetus

Special Problems in Pregnancy

Vomiting

1. Common symptom up to 16 weeks of gestation
2. Assess current status: Hydration, pulse rate, blood pressure
3. Check for other causes: Jaundice, neck stiffness, abdominal tenderness / guarding
4. If no dehydration & urine ketone negative:
 - Advice: Small frequent meals; avoid cooking smells; fizzy drinks may help
 - Treatment: Pyridoxine 10-25 mg TDS, Promethazine 1 tab TDS or Metoclopramide 10 mg TDS
5. If dehydrated, urine ketone positive or suspect underlying medical or surgical conditions: Refer to hospital

Anaemia

1. If Hb < 9 g/dL: Refer to hospital
2. If anemia is microcytic / hypochromic: Exclude blood loss, check serum ferritin and Hb electrophoresis
 - Consider FBC and Thalassemia screening in spouse
 - Refer to Obstetric for all pregnant women with newly diagnosed Thalassemia
3. If anemia is macrocytic: Check B12 and folate levels
4. Prescribe haematinics if iron deficient

Urinary Problems

1. Perform UFEME and urine culture if urine dipstick shows positive leucocytes or nitrite
 - Asymptomatic bacteriuria in pregnancy must be treated as it is associated with significant risks
2. Dysuria: Perform UFEME and urine culture to confirm UTI, exclude pyelonephritis
3. Frequency: Assess fluid intake, exclude DM and UTI. If all the above are normal, consider the possibility of pelvic tumour
4. Proteinuria: Consider causes including UTI, hypertension, renal disease, contamination and orthostatic (by exclusion)

Backache

1. Usual features of backache in pregnancy: Diffuse, no motor or sensory impairment
2. Red flags: Non mechanical, associated with systemic symptoms e.g. fever, symptoms of nerve root compression
3. Treatment: Symptomatic relief

Elevated Blood Pressure

1. Blood pressure measurement should be done at each visit
2. Pregnancy induced hypertension occurs after 20th week of gestation
3. Refer to hospital SOC if:
 - BP $\geq$ 140/90 mmHg, or
 - A rise of systolic BP $\geq$ 30 mmHg or diastolic BP $\geq$ 15 mmHg from previous readings (2 readings taken 4 hours apart)
 - Start anti-hypertensives such as Nifedipine, labetalol or methyldopa if indicated
4. In the presence of elevated blood pressure, refer to ED if the following red flags occur:

- Severe hypertension (systolic BP $\geq$ 160 mmHg or diastolic BP $\geq$ 110 mmHg on two occasions at least four hours apart or only once if treated), persistent and/or severe headache, visual abnormalities, upper abdominal or epigastric pain, nausea and vomiting, dyspnoea, retrosternal chest pain, altered mental status, seizures
5. For pre-eclampsia prophylaxis, to start aspirin 100 mg every NIGHT from 12-16 weeks onwards until 36 weeks if the woman has two or more moderate risk factors or one high risk factor:

Moderate risk factors:

- Age 40 years or older
- Nulliparity / First pregnancy
- New partner
- Pregnancy interval of more than 10 years
- BMI of 35 kg/m² or more

High risk factors:

- Previous early onset (< 34 weeks) pre-eclampsia or gestational hypertension
- Pre-existing hypertension
- Pre-existing kidney disease
- Diabetes Mellitus
- Anti-phospholipid syndrome, SLE

Abnormal 2-point OGTT (75g oral glucose load) in 1st trimester of pregnancy

1. Pre-existing diabetes should be diagnosed by the 2006 WHO criteria for diabetes if one or more of the following criteria are met:
 - Fasting venous plasma glucose $\geq$ 7.0 mmol/L
 - 2-hour venous plasma glucose $\geq$ 11.1 mmol/L
 - Random venous plasma glucose $\geq$ 11.1 mmol/L in the presence of diabetes symptoms
2. Pre-diabetes should be diagnosed if one or more of the following criteria are met:
 - Fasting venous plasma glucose 6.1 - 6.9 mmol/L
 - 2-hour venous plasma glucose 7.8 - 11.0 mmol/L
3. The above conditions should be referred to Obstetrics for management.
4. If fasting venous plasma glucose 5.1 - 6.0 mmol/L, and 2-hour venous plasma glucose < 7.8 mmol/L, refer to dietician for healthy diet advice

Abnormal 3-point OGTT (75g oral glucose load) in mid-gestation

1. Gestational diabetes mellitus should be diagnosed if one or more of the following criteria are met (IADPSG 2010):
 - Fasting venous plasma glucose 5.1 - 6.9 mmol/L
 - 1-hour venous plasma glucose $\geq$ 10.0 mmol/L*
 - 2-hour venous plasma glucose 8.5 - 11.0 mmol/L
2. Diabetes in pregnancy should be diagnosed by the 2006 WHO criteria for diabetes if one or more of the following criteria are met:
 - Fasting venous plasma glucose $\geq$ 7.0 mmol/L
 - 2-hour venous plasma glucose $\geq$ 11.1 mmol/L
 - Random venous plasma glucose $\geq$ 11.1 mmol/L in the presence of diabetes symptoms
3. The above conditions should be referred to Obstetrics for management.

Others

1. Vaginal discharge
 - Physiological vaginal discharge is mucoid, white or cream in colour
 - Not associated with itch, not foul-smelling
2. Bronchial asthma
 - Management follows that of non-pregnant patients
3. Drugs
 - Certain drugs are contraindicated or to be used with caution
 - Check from the formulary book if unsure
4. Vaccination
 - Live vaccines are contraindicated. These include MMR, varicella, BCG, Poliomyelitis, Smallpox
 - Influenza (inactivated) may be given at any trimester but ideally before 3rd trimester as the risk of flu is highest during that period
 - Tdap - unvaccinated pregnant women should get one dose of Tdap between 20 to 28 weeks gestation
 - Hepatitis B - not contraindicated but administered usually postpartum
 - HPV - if inadvertently given 1st dose, to complete the three-dose series postpartum

POSTNATAL CARE (AT 6 WEEKS AFTER DELIVERY)

Symptoms and Signs	Management	Referral	Symptoms and Signs	Management	Referral
History Taking <u>Obstetric Issues</u> - Parity - Date of delivery - Mode of delivery - Antenatal and postpartum issues <u>Screen for Common Postnatal Problems</u> - Postnatal depression - Enquire regarding patient's general well-being and coping mechanisms - Screen mood - Home support system - Bowel issues - Constipation - Haemorrhoids - Faecal incontinence - Urinary Issues - Urinary incontinence - Dysuria - Pain - Abdominal cramps - Episiotomy / Caesarean wound - Lochia - Amount and colour of lochia	Refer to memo and discharge summary from hospital for further information or special instructions Consider using Edinburgh Postnatal Depression Scale to screen Encourage high fibre diet Laxatives if required Exclude urinary tract infection and other reversible causes Determine cause of pain Analgesia if required Watch for secondary postpartum haemorrhage	Refer to NUH Women Emotional Health Service (WEHS) Refer to colorectal surgeon for haemorrhoidectomy if needed For urodynamic studies if incontinence persists after 6 weeks despite pelvic floor exercises Refer back to gynae if persistent PV bleed suspicious of secondary PPH	- Breastfeeding Issues - Latching problems, sore nipples - Breast engorgement, breast lumps - Over/under supply - Fever - Common sources: - Wound - Breast (mastitis, abscess) - Uterus (endometritis, retained products of conception) - UTI <u>Check regarding New-born</u> - General wellbeing, weight gain - Medical problems, e.g. NNJ, feeding issues - Hospital follow up - Immunizations, developmental checks - Main caregiver - Coping <u>Family Planning</u> - LMP - Previous contraception used <u>Preventive Care</u> - Pap smear - Vaccination	Breastfeeding advice Determine source of fever and manage accordingly Discuss with patient on family planning options Advise on possibility of pregnancy, even with exclusive breastfeeding Schedule next Pap smear accordingly as per CSS guidelines Check antenatal test results and arrange for relevant vaccinations	Engage lactation consultant for further support if necessary Refer breast surgery if breast abscess present Refer back gynae if suspicious of uterine sources of fever

Symptoms and Signs	Management	Referral
Physical examination		
<u>Parameters</u> Blood pressure	Watch for cases of pre-existing essential hypertension, pregnancy induced hypertension and pre-eclampsia	
Body weight		
Temperature • Puerperal fever defined as temp > 38°C on two occasions after first 24 hours postpartum	Identify possible causes: • Endometritis • Wound infection • Breast engorgement • Urinary Tract Infection • Venous thromboembolism (VTE)	Refer for treatment of puerperal fever / infection Refer if VTE is suspected with unilateral calf swelling and tenderness
<u>General Status</u> Hydration Pallor Fatigue		
<u>Breast Examination</u> Check for: • Breast engorgement • Mastitis • Breast abscess	Consider antibiotics for treatment of mastitis	Refer to breast surgeon for breast abscess

Symptoms and Signs	Management	Referral
<u>Abdominal Examination</u> Palpate uterine size and check for tenderness	Manage surgical wound, watch for wound infection or breakdown	Refer to O&G if signs of wound infection and breakdown
Examine surgical scar in post-Caesarean cases		
<u>Pelvic Examination</u> • Perineum: check episiotomy site	Assess for signs of wound infection or breakdown	Refer to O&G if signs of wound infection and breakdown
• Speculum: check colour and amount of lochia, check cervix	Assess for secondary postpartum haemorrhage	Refer to O&G for secondary postpartum haemorrhage
• Bimanual palpation: assess uterine size and watch for tenderness, adnexal tenderness or masses	Arrange for pap smear as per CSS guidelines	
<u>Rectal Examination</u> Check for anal fissures and haemorrhoids	Symptomatic management if necessary	
<u>Limbs</u> Watch for signs of DVT		

Others:

Manage pre-existing chronic illnesses, e.g. diabetes, hypertension, asthma

For patients with GDM, repeat 2-point OGTT 6 weeks post delivery

To continue follow up with yearly DM screen thereafter

For patients with hypertensive disorders in pregnancy (pre-eclampsia or pregnancy-induced hypertension) - if still hypertensive at 6 weeks post-delivery, need to exclude underlying medical conditions (e.g. renal, endocrine causes).

If none, to continue follow-up as life-time risk of cardiovascular disease is increased 2-3 fold.

Offer lifestyle advice (e.g. weight loss, low salt diet) and regular BP checks.

Breastfeeding

The nursing mum's milk will become plentiful or "come in" sometime between the second and sixth day after giving birth. It takes two weeks for mum's milk to gradually transition into mature milk.

The effects of engorgement can be prevented or minimized by the following:

- Nurse early, preferably immediately after birth and at least ten times a day.
- Ensure that the baby is well positioned and latched on properly.
- Nurse "on cue". If the baby sleeps more than two to three hours during the day or four hours at night, wake him to nurse.

Additional suggestions for dealing with breast engorgement

1. Gentle breast massage

With the palm of the hand and starting from top to the chest, gentle stroke the breast downward in a circular motion, towards the nipple. This may be more effective when done while in the shower or while leaning over a basin of warm water and splashing water over the breast.

2. Warm compression, massage, cold compression

Some mothers find that applying a warm, moist compress and expressing some milk just before feeding helps to relieve engorgement. Using heat for too long will increase swelling and inflammation; hence it is best to keep it brief. Cold compresses can be used between feeds to reduce swelling and pain.

3. Cabbage compresses

A popular home remedy for relieving the discomfort of engorgement is cabbage leaf compresses. Rinse the inner leaves of a head of cabbage, remove the hard vein, and crush with a rolling pin (or similar). They can be used refrigerated or at room temperature. Drape leaves directly over breasts, inside the bra. Change when the leaves become wilted, or every two hours. Discontinue use if rash or other signs of allergy occur. There have been reports that overuse of cabbage compresses can reduce milk production; therefore, some experts suggest mothers discontinue the compresses when the swelling goes down.

4. Lactation consultants are available at each hospital to help new mothers with issues related to lactation and breastfeeding. Patients may contact them directly through the hospital.

5. The use of medications (Dostinex, Bromocriptine) for suppression of postpartum lactation is not well studied and should be used with caution. Please prescribe as external prescription if needed.

- For inhibition of lactation: Dostinex 1 mg given as a single dose on first day postpartum
- For suppression of established lactation: Dostinex 0.25 mg every 12 hours for 2 days

POSTPARTUM DEPRESSION (PPD)

Definition

Major depressive disorder with onset during the “peripartum period”.

The Diagnostic and Statistical Manual, Fifth Edition defines this as depression with onset during pregnancy or in the 4 weeks following delivery, while the World Health Organisation's International Classification of Diseases -10th Revision defines this as within 6 weeks following delivery.

However, clinicians should note that symptoms may be unnoticed initially and patients may only present later, up to **1 year post-delivery**¹.

In Singapore, the prevalence of peripartum depression is about 12% for antepartum depression, and about 7% for postpartum depression².

Difference between PPD and baby blues

'Baby blues' can occur in up to 80% of mothers following delivery. Its onset is in the first 2 to 3 days after delivery. Symptoms are typically mild and resolve within 2 weeks. On the other hand, postpartum depression persists for more than 2 weeks and causes moderate to severe dysfunction. Suicidal ideation may also be present³.

Risk factors²

- History of depression or other mental illnesses
- Lack of adequate support from family and friends
- Unplanned pregnancy
- Financial, work or relationship problems
- Pregnancy and labour complications
- Negative confinement experience
- Teenage pregnancy⁴

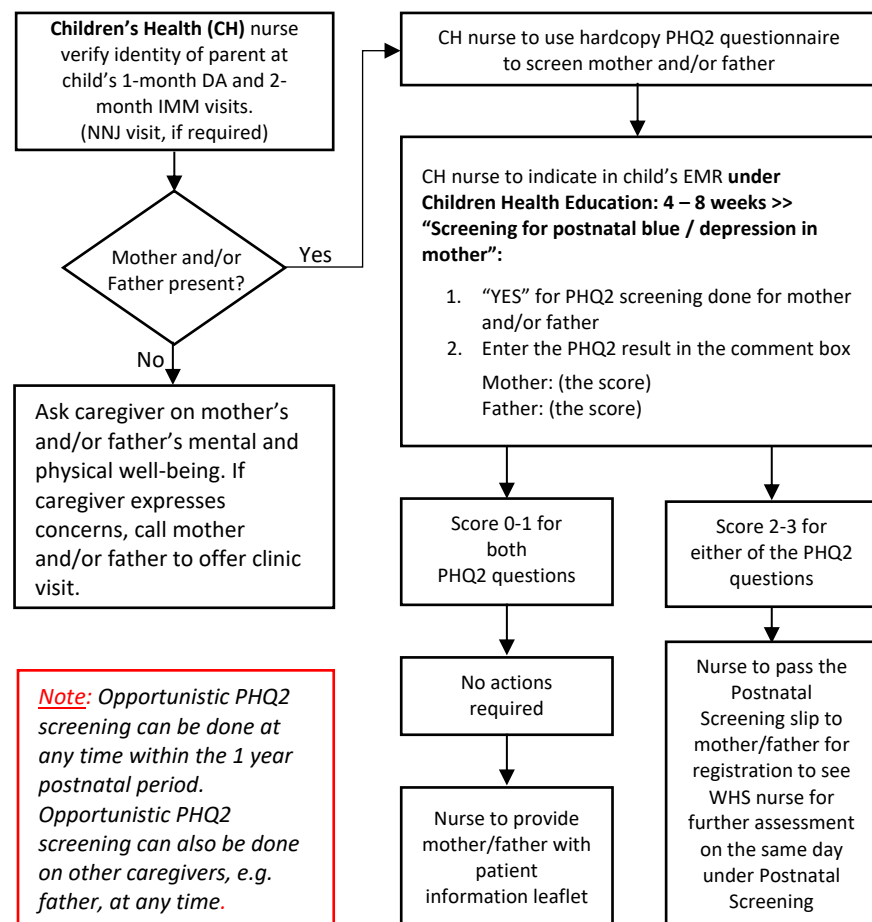
Assessment

- Use the Edinburgh Postpartum Depression Scale ([Annex B](#)) or Patient Health Questionnaire-9⁵
- Ascertain the diagnosis and severity
- Identify top 3 stressors
- Identify coexisting psychiatric disorders
- Exclude prior episodes of mania or hypomania
- Assess suicide and homicide risk
- Exclude organic causes e.g. hypothyroidism
- Assess family / social support

Screening for postpartum depression by nurses

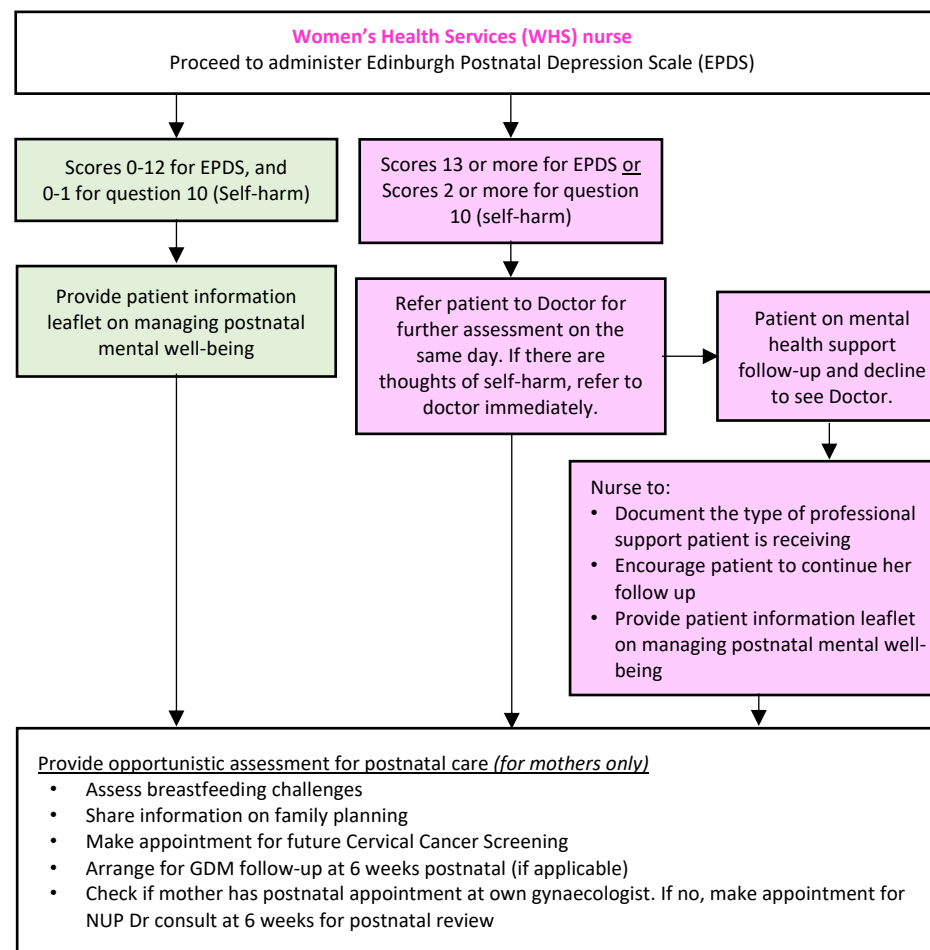
During the 1-month DA and 2-month IMM visits of babies, if they are accompanied by their mothers, the Children's Health nurse will offer to administer a PHQ-2 screen for the mother. If the screen is positive (taken to be score of 2 or more for either question), she will be registered to see the Women's Health nurse, who will administer the Edinburgh Postnatal Depression Scale (EPDS - [Annex B](#)) for her. If the score is 13 or more, she will be referred to the doctor. (see flowchart below)

Workflow for Postnatal Depression Screening



Note: Opportunistic PHQ2 screening can be done at any time within the 1 year postnatal period. Opportunistic PHQ2 screening can also be done on other caregivers, e.g. father, at any time.

Note: If WHS nurse is occupied and patient does not have time to wait, consider registering mother / father directly to doctor for further assessment.



Management by doctors

For patients who score 13 or higher on the EPDS, and are referred to see the doctor, assess the patient clinically and formally confirm the diagnosis of postpartum depression (PPD). Patients who have been diagnosed with PPD should be referred to the psychiatrist for further management. (see table below)

Postpartum duration	Criteria	When and Where to refer
Within 1 year	Active suicidal thoughts Thoughts of harming self, newborn or others Psychosis	Emergency Department on same day for evaluation and possible inpatient treatment
	New-onset postpartum depression or anxiety - EPDS score ≥ 13 - Positive score for question 10 (2 or more)	Direct Access referral (within 2 weeks) • NUH Women's Emotional Health Service • KKH Women's Mental Wellness Service
	Postpartum depression with - Past or present severe mental illness (including schizophrenia, bipolar disorder, psychosis in the postnatal period and severe depression) - Pre-existing depression - Family history of maternal perinatal mental illness	
After 1 year	Symptoms of depression	Management would be as per any patient with depressive symptoms. If requiring specialist referral, refer to NUHS General Psychiatry.

Referral to NUH Women's Emotional Health Service

1. Refer to Psychological Medicine – NUHS
2. Under “Main Reason for Referral”, select “Perinatal (Pregnant and postnatal up to 1yr)”
3. Under “Patient's Preferred Institution”, select NUH WEHS

REFERRAL TO PSYCHOLOGICAL MEDICINE - NUHS✓ Accept✗ Cancel

Main Reason for Referral

Mood Disorder

Anxiety

PTSD

OCD

Insomnia

Psychosis

Capacity Assessment

Addictions

Comments

Memory Issues

Perinatal (Pregnant and postnatal up to 1yr)

Somatic Symptom Distress

Eating Disorder

Forensic Issues

Others (Please Specify)

Does the patient have a past psychiatric follow-up with a public hospital within the last 10 years?

Yes

No

Patient Language

English

Mandarin

Malay

Tamil

Cantonese

Hokkien

Others

Referral Type? :

Subsidized referral if eligible

Non-subsidized referral

Scheduling Instructions

Direct Access (<=2 weeks)

Routine (<=8 weeks)

OPTIONAL - Patient's Preferred Institution

NUH-WEHS

Process Inst.: For clarifications, please call: Psychiatry consultant on call at each site via Call Centre

Immediate referral to IMH
1. Patients at high risk of harm to self or others

Immediate referral to ED
1. Patients at high risk of harm to self or others but need medical clearance

Comments:

⊕ abc ↶ ↷ ? ⚡ +

Insert SmartText

⬅ ➡ ↶ ↷

Diagnoses:
The encounter diagnosis was X.

Active Problem List:

ⓘ Next Required✓ Accept✗ Cancel

Referral to KKH Women's Mental Wellness Service

1. Refer to Psychiatry – Non Epic
2. Indicate “KK Women's and Children Hospital under “Referral”
3. Write down “Women's Mental Wellness Service” under “Sub-specialties”.
4. Under “Reason for referral”, indicate “Postpartum Depression”.
5. Remember to copy and paste the Dr's notes into the referral.

REFERRAL TO PSYCHIATRY - NON-EPIC✓ Accept ✗ Cancel

Referral: To loc/pos:

Sub-specialties

Referral to (Person) :

Reason for referral

Referral Type? :

Scheduling Instructions

Status:

Expected Date:

☐ Approx.

Comment:

Expires:

Comments:

Copy and paste the Dr's notes into the referral

Next Required✓ Accept ✗ Cancel

If patient declines referral

1. Reinforce importance of appropriate management and monitoring of PPD, and offer referral again.
2. If patient still declines, provide patient information brochure on managing postnatal mental well-being
3. Provide a follow-up to review in 2 weeks.

Management guidelines

- **Mild (EPDS score ≤ 13)⁶:** Start with Non-pharmacological methods⁷
 - Mental health advice on self-care, Support from partner / family / friends
 - Educate mothers on parenting skills and caring for their infant, lactation support
 - Share resources for mental wellness and safety netting. Reach out to helplines or counselling services, Family Service Centres.
 - **Consider referring** patient to MSW to discuss support needed / available. Refer to Medical Social Services-Financial Counselling for any financial concerns.
 - Refer to psychologist for Psychotherapy if patient agrees
- **Moderate to Severe (EPDS score > 13):** Consider starting pharmacological interventions and non-pharmacological interventions as above
 - SSRIs as the drug class of choice
 - SSRIs safe in lactation: Fluvoxamine 25-50 mg ON; Sertraline 25-50 mg OM; Escitalopram 5-10 mg OM⁸
 - Side effects: GI, Nausea, abdominal pain, diarrhoea, constipation; headache; dry mouth; sexual dysfunction
 - Can consider TCAs if suicide risk is low and patient has prominent issues with poor sleep: Dothiepin 25-50 mg, Amitriptyline 10-25 mg ON

Follow-up^{7, 9}

- After starting SSRIs, review the patient in 2-4 weeks: review their symptoms, drug tolerability/ side effects and suicide risk.
 - If there is a partial response after 2 weeks, consider increasing dosage for better response.
 - If there is no response after 4 weeks, consider a switch within the class.
- In the first 1-2 months of starting antidepressants, monitor the patient for the emergence or worsening of suicidal thinking and unusual changes in behaviour. This is particularly important for young adults between 18 to 24 years old.
- Stop the antidepressant if there are hypomanic or manic symptoms and refer to a psychiatrist
- Discuss future pregnancy plans
 - SSRIs have low teratogenicity, and are not definitively associated with malformations in large scale studies. Apart from paroxetine (which is associated with increased risk of cardiac malformation), data is limited but reassuring.
 - TCAs are not associated with malformations.
 - If necessary for the patient's treatment of depression, pregnancy-safe SSRIs may be continued antenatally as the risks to mother and foetus of untreated maternal depression still outweigh the risks associated with antidepressants.
 - Nevertheless, it may be prudent for the patient to consider family planning to avoid the extra stress of pregnancy and caring for another newborn while she is still unwell.
- Discontinuation of antidepressants:
 - Discontinuation of antidepressants can be discussed if it is the patient's 1st depressive episode and they have responded to medicine for at least **6-9 months after remission of symptoms**. You should also consider the patient's current psychosocial situation and readiness to stop medication.
 - Tail down the antidepressant gradually over 4 weeks or more to reduce the risk of discontinuation symptoms.
 - Follow up the patient for at least 6 months for recurrence. This is the period during which relapse is the highest.

MENOPAUSE

Overview

1. In general, natural menopause occurs between the ages of 45 and 55 years.
2. The perimenopause period is the time from the onset of cycle irregularity through until 12 months after the menstrual period. May last for up to 4 years.
3. Surgical menopause is the removal of both ovaries.
4. Primary ovarian insufficiency (POI) is cessation of ovarian function before the age of 40 years.

Symptoms

1. Vasomotor
 - Hot flushes
 - Night sweats
 - Palpitations
2. Psychological
 - Depressive symptoms
 - Anxiety, irritability
 - Sleep disturbance
 - Overall diminished well-being
 - Reduced memory
3. Physical
 - Sleep disturbance
 - Fatigue
 - Headaches
 - Muscle and joint pain
 - Crawling sensations on skin (formication)
4. Urogenital and sexual
 - Vaginal itching, burning
 - Dryness and dyspareunia
 - Urinary frequency, urgency
 - Reduced libido
5. Long term implications
 - Osteoporosis
 - Cardiovascular disease
 - Dementia
 - UTI, prolapse
 - Sexual dysfunction

Diagnosis

The diagnosis of menopause is mostly straightforward if a woman is over the age of 45 years and reports cessation of menstruation for over 12 months, with or without symptoms.

Challenging situations, in terms of diagnosis, include:

- Women who have had an endometrial ablation
- Women who have a progestin-releasing IUCD
- Women who are using systemic hormonal contraception (oestrogen plus progestin oral, transdermal or vaginal ring or implanted, depot progestin)
- Women who have symptoms and are less than 45 years old

Investigations

Most women do not need to have any hormonal tests done to diagnose menopause.

Hormone measurement may be useful for:

- Women with subtle or fluctuating symptoms, e.g. predominant mood change and few vasomotor symptoms
- Women younger than 45 years old
- Women without a uterus
- Women on progesterone only contraception that renders them amenorrhoeic
- To diagnose primary ovarian insufficiency (POI): Diagnosis requires FSH to be elevated and E2 to be low on at least 2 occasions, at least 4 to 6 weeks apart

Other investigations can be considered, depending on clinical assessment:

- Exclude other causes of amenorrhea in younger women
 - Pregnancy - Urine HCG
 - Hyperprolactinemia - Serum prolactin
 - Thyroid disease - Thyroid function test
 - Hypothalamic amenorrhea (anorexia nervosa, *etc.*)
- Exclude other common causes of fatigue, mood change, hotness
 - Thyroid disease - Thyroid function test
 - Iron deficiency - Haemoglobin, iron stores
 - Type 2 Diabetes - Fasting venous glucose, HbA1c

Management

Mild to moderate symptoms

Symptoms	Management
Vasomotor symptoms e.g. Hot flushes, night sweats	• Exercise regularly • Have cool showers and stay in cooler rooms with adequate ventilation • Reduce intake of caffeine, alcohol, hot beverages and spicy foods. • Wear thin cotton clothes • Encourage consumption of soy products like tofu and soya bean
Disrupted sleep	• Keep room cool with circulating fresh air. • Good sleep hygiene • Exercise and relaxation also improve sleep
Muscle and joint pain	• Symptomatic treatment, e.g. painkillers • Try gentle, non-weight bearing exercise such as cycling or swimming • Losing weight can reduce the load on the joints
Urinary symptoms	• Crossing the legs when you feel a cough or sneeze starting can help prevent leakage. • Avoid constipation
Vaginal dryness	• Water-soluble vaginal lubricants can reduce dryness and make sex more comfortable • Avoid excessive wash with soap • Consider vaginal oestrogen therapy, e.g. Premarin cream, Vagifem
Mood changes e.g. Irritability, anxiety, forgetfulness, loss of concentration, low self-esteem, depression	• Exercise regularly • Take up a hobby • Talk to friends, family members

Moderate to severe symptoms

Refer to Menopause clinic for:

1. Hormonal treatment
2. Non-hormonal treatment
3. Complex medical conditions
4. Premature ovarian insufficiency
5. Early menopause

Evidence-based non-hormonal treatment for vasomotor symptoms

- Selective serotonin reuptake inhibitors (SSRI) and selective serotonin-norepinephrine reuptake inhibitors (SNRI)
- Clonidine
- Gabapentin
- Pregabalin
- Hypnosis
- Cognitive behaviour therapy
- Weight loss for obese women
- Stellate ganglion blockade (for severe resistant symptoms)

Hormone Replacement Therapy (HRT)

HRT is the most effective treatment of menopause-related vasomotor symptoms and its consequences, such as poor sleep, irritability, poor quality of life.

For patients who verbalize moderate to severe symptoms which affect their quality of life, refer to Menopause clinic for consideration of initiation of HRT.

Contraindications to oral HRT:

- Breast cancer
- Endometrial cancer (not if hysterectomy and early stage)
- Ischaemic heart disease*
- Previous venous thromboembolic event*
- Previous stroke / transient ischaemic attack*
- Active liver disease*
- Unexplained vaginal bleeding

Risks of oral HRT:

- Slightly increased risk of
 - Breast cancer
 - Stroke
 - Venous thromboembolism
 - Coronary heart disease (if started more than 10 years after menopause or above age of 60 years old)

** Patients with these conditions may be suitable for transdermal HRT. There is also no increased risk of stroke or venous thromboembolism with the use of transdermal HRT.*

Patients with significant menopausal symptoms should be counselled carefully regarding benefits and risks of HRT, and misconceptions should be clarified and cleared.

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ABBREVIATIONS

<	Less than
>	More than
≥	More than or equal to
BP	Blood pressure
CDC	Communicable Disease Centre
DM	Diabetes Mellitus
DSC	Department of Sexual Disease Control
ED	Emergency Department
EDD	Estimated delivery date
FBC	Full blood count
GDM	Gestational Diabetes Mellitus
Hb	Haemoglobin
HIV	Human Immunodeficiency Virus
IADPSG	International Association of Diabetes and Pregnancy Study Group
LMP	Last menstrual period
MCV	Mean corpuscular volume
OGTT	Oral glucose tolerance test
SOC	Specialist Outpatient Clinic
Thal	Thalassemia
UFEME	Urine full examination microscopic examination
UPT	Urine pregnancy test
UTI	Urinary tract infection
VDRL	Venereal Disease Research Laboratory

ANNEX

Annex A: Criteria for Excluding Pregnancy

(adapted from UK Selected Practice Recommendations for Contraceptive Use)

Health professionals can be 'reasonably certain' that a woman is not currently pregnant if any one or more of the following criteria are met and there are no symptoms or signs of pregnancy:

- Has not had intercourse since last normal menses
- Has been correctly and consistently using a reliable method of contraception
- Is within the first 7 days of the onset of a normal menstrual period
- Is within 4 weeks postpartum for non-lactating women
- Is within the first 7 days post-abortion or miscarriage
- Is fully or nearly fully breastfeeding, amenorrhoeic, and less than 6 months postpartum.

A pregnancy test, if available, adds weight to the exclusion of pregnancy but only if ≥ 3 weeks since the last episode of unprotected sexual intercourse.

NB. Health professionals should also consider if a woman is at risk of becoming pregnant as a result of unprotected sexual intercourse within the last 7 days and undertake pregnancy testing where appropriate (≥ 3 weeks since last unprotected sexual intercourse).

Source: Faculty of Sexual & Reproductive Healthcare Clinical Guidance. Combined Hormonal Contraception. (2012).

Retrieved from <http://www.fsrh.org/pdfs/CEUGuidanceCombinedHormonalContraception>

Annex B: Edinburgh Postnatal Depression Scale (EPDS)



Edinburgh Postnatal Depression Scale¹ (EPDS)

Total Score: _____

Date: _____

Name: _____ ID/NRIC No: _____

Date of Birth: _____ Baby's Date of Birth: _____

As you are pregnant or have recently had a baby, we would like to know how you are feeling.

Please cross (X) the answer that comes closest to how you have felt IN THE PAST 7 DAYS, not just how you feel today.

Here is an example, already completed.

I have felt happy:

- ☐ Yes, all the time
☐ Yes, most of the time
☐ No, not very often
☐ No, not at all

This would mean: "I have felt happy most of the time" during the past week. Please complete the other questions in the same way.

In the past 7 days:

1. I have been able to laugh and see the funny side of things

- ☐ As much as I always could
☐ Not quite so much now
☐ Definitely not so much now
☐ Not at all

2. I have looked forward with enjoyment to things

- ☐ As much as I ever did
☐ Rather less than I used to
☐ Definitely less than I used to
☐ Hardly at all

***3. I have blamed myself unnecessarily when things went wrong**

- ☐ Yes, most of the time
☐ Yes, some of the time
☐ Not very often
☐ No, never

4. I have been anxious or worried for no good reason

- ☐ No, not at all
☐ Hardly ever
☐ Yes, sometimes
☐ Yes, very often

***5. I have felt scared or panicky for no very good reason**

- ☐ Yes, quite a lot
☐ Yes, sometimes
☐ No, not much
☐ No, not at all

***6. Things have been getting on top of me**

- ☐ Yes, most of the time I haven't been able to cope at all
☐ Yes, sometimes I haven't been coping as well as usual
☐ No, most of the time I have coped quite well
☐ No, I have been coping as well as ever

***7. I have been so unhappy that I have had difficulty sleeping**

- ☐ Yes, most of the time
☐ Yes, quite often
☐ Not very often
☐ No, not at all

***8. I have felt sad or miserable**

- ☐ Yes, most of the time
☐ Yes, quite often
☐ Not very often
☐ No, not at all

***9. I have been so unhappy that I have been crying**

- ☐ Yes, most of the time
☐ Yes, quite often
☐ Only occasionally
☐ No, never

***10. The thought of harming myself has occurred to me**

- ☐ Yes, quite often
☐ Sometimes
☐ Hardly ever
☐ Never

¹Cox, J., Holden, J., & Sagovsky, R. (1987). Detection of postnatal depression: Development of the 10-item Edinburgh Postnatal Depression Scale. *British Journal of Psychiatry*, 150(6), 782-786. <https://doi.org/10.1192/bjp.150.6.782>